

Our Ref: (T) //[MED-ID]

[DATE]

Clinic Letter : [DATE]  
[PROVIDER-NAME]  
Waterside Medical Pract  
Elm Grove, Mengham  
[ADDRESS]  
[ADDRESS]  
[POSTCODE]

Division of Women & Children  
[ADDRESS]  
[ADDRESS]  
[ADDRESS]  
[ADDRESS]  
[POSTCODE]

Telephone: [PHONE] Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME], [ADDRESS] [POSTCODE]  
DOB: [DATE], Hospital Number: [MED-ID], NHS Number: [NHS-NO]

Problem: ? ulcerative colitis

I received a referral from [PROVIDER-NAME] to see [NAME]. [NAME] is a 14-year-old girl with a history of intermittent abdominal pain associated with the need to pass stool. [NAME] has caused [NAME] a lot of time off school particularly mornings. Her symptoms have come to her head in the last couple of months and were particularly bad during the Easter holidays. During [NAME] time she was passing a bloody and mucousy stool up to six to seven times daily. [NAME] was associated with abdominal pain that prevented her from doing any usual activities. She was admitted to Portsmouth Hospital on 16 April and I started her on metronidazole and mesalazine. Since then she has felt much better and is now passing two or three stools a day with occasionally a trace of blood. She still has intermittent abdominal pain mainly associated with the need for stool. She has a feeling of urgency but no longer any incontinence.

[NAME] has been having trouble at school over the last year with bullying. She does not have many friends but she has developed strategies to cope with the bullying and is planning to go back next week. She is currently excluding some gluten and dairy from her diet although not completely. She reports no recent weight loss. She started her periods [NAME] year but these are erratic and she has had two or three so far.

[NAME] was born by a normal delivery at term. She tried a variety of standard formula milks but

settling on one. [NAME] were no problems around weaning. She has had tonsils and adenoids removed. [NAME] had grommets but no other significant medical history. She has no allergies. [NAME]'s mother has ulcerative colitis diagnosed at a similar age. [NAME] lives with her nan who is her legal guardian. [NAME] grand dad. [NAME] has lived with her nan since the age of 5. [NAME] sees her mum regularly but has no contact with her father.

On examination today [NAME]'s height was 161.2 cm (50th centile) and weight 56.2 (75th centile). [NAME] looked well, had no finger clubbing. Cardiovascular and respiratory examination were unremarkable. [NAME] abdomen was soft and mildly tender in the left flank.

Summary and Plan: [NAME] is a girl who has a history compatible with ulcerative colitis. I have taken [NAME] her bloods today and will chase up the result of her ultrasound scan from Portsmouth. We have booked [NAME] for endoscopy next week.

Yours sincerely  
[PROVIDER-NAME]  
Paediatric Registrar to [PROVIDER-NAME] to:

[ADDRESS]

[POSTCODE]